

**STRATEGY & OPERATIONS**

Vytalix Investor Proposition

Vytalix company-build documentation · reference

10_INVESTOR_PROPOSITION

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

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Owner: Fundraising Director · **Status:** PROPOSED v1 (8 September 2026) · **Governing file:** /FACTS_BASE.md ·
Numbers from: /docs/08_FINANCIAL_MODEL.md · **Rounds and gates from:**
 /docs/09_FUNDING_STRATEGY.md · **Derived assets:** /assets/INVESTOR_ONE_PAGER.md ,
 /assets/INVESTOR_PITCH_DECK.md

Truth standard. This is the narrative and answer-sheet for investor conversations that may begin only after the pre-conditions in 09_FUNDING_STRATEGY §8 are met (entity, IP heads of terms, claims retired, data room, SEIS/EIS applied for). Vytalix has no revenue, customers, pilots, partners, grants, approvals or team beyond the founder (KNOWN). Every number is a scenario or an ESTIMATE; every market figure is labelled; sources are cited only where already verified in /docs/05_MARKET_STRATEGY.md . MaternaLink is "in development" in every sentence. GBP throughout.

Executive view

- The pitch in one line:** Vytalix is a UK health-technology group that funds itself with founder-led advisory and consulting while building MaternaLink (in development) — a non-diagnostic maternal care-coordination platform for the NHS and low-connectivity settings — and it asks for a £400k SEIS/EIS pre-seed to turn a concept and a prototype into an owned, safety-documented, design-partnered product.
- Why now, honestly:** UK maternity inquiries keep finding the same failures to listen, escalate and hand over; MBRRACE-UK 2025 reports Black women more than twice as likely to die as White women; Nigeria carries the highest maternal mortality ratio in Africa. No incumbent owns the coordination-and-handover layer beside the record — EPR vendors own the record, charities own information, donor programmes own messaging.
- What an investor is buying at pre-seed** is not the FY2029 P&L (Base revenue £820k, 34% MaternaLink) but the milestones that make MaternaLink a licensable asset: IP inside the company, safety case, DPIA, DTAC readiness, an MVP, a UK design partner and a service-evaluation protocol — each is a seed-round gate and each is dated.
- Traction is stated as what exists, what is being built and what is next** — with 90-day and 180-day proof points and the evidence each will produce. Today: a concept deck, an unaudited prototype, one unsolicited government proposal of unknown outcome, and a 300-investor CRM. That is the whole truth.
- Exit language is deliberately modest:** plausible acquirers are maternity-record vendors, women's-health platforms, health-system consultancies and impact-oriented strategics — described as types, with no timing or multiple promised. The founder's commitment is to build a company that is worth owning first.

1. The pitches

1.1 One line

Vytalix funds itself with health-technology advisory and consulting while building MaternaLink (in development): the coordination and handover layer beside the maternity record, designed for NHS trusts and low-connectivity settings alike — safety-first, evidence-first, no clinical claims until evidence exists.

1.2 Thirty seconds

"Maternal care fails most often between people — between a woman and a changing team, between shifts, between a clinic and a hospital. UK inquiries keep finding the same failures to listen, escalate and hand over, and in Nigeria the delays are measured in lives. Vytalix is a UK health-tech group. Our advisory and consulting practice

pays the bills from month two. Our product, MaternaLink — in development — is a non-diagnostic care-coordination platform: enrolment, reminders, signed-off education in the woman's language, two-way messaging with a named human owner, structured handover, escalation log. Same codebase for a London trust and a rural Nigerian LGA. We're pre-seed, pre-formation, and we say so. We're raising £400k under SEIS/EIS to own the IP, complete the safety artefacts, build the MVP and win a UK design partner — the gates a seed investor will ask for."

1.3 Two minutes (script; ~300 words)

"The problem. Every UK maternity inquiry of the last decade — Ockenden, Kirkup, the current national investigation — finds the same thing: failures to listen, escalate and hand over. MBRRACE-UK's 2025 report shows Black women more than twice as likely to die around childbirth as White women. In Nigeria, the maternal mortality ratio is the highest in Africa, and most of that is delay — in seeking, reaching and receiving care. These are coordination failures before they are clinical ones.

The gap. The UK maternity record is a three-vendor oligopoly built around the record, not the woman. Charity apps give information, not coordination. African programmes run excellent messaging at under a dollar a mother, but nobody owns facility-side coordination and handover. That layer is empty.

What we're building. MaternaLink — in development — is that layer: a non-diagnostic care-coordination, communication, education and monitoring-support platform. It does not diagnose, predict or score. It is designed to make sure a woman is connected to a named person who has to respond, and that the next professional knows what the last one knew. Offline-first, SMS and voice as well as web, so one product serves an NHS trust and a rural local-government area.

How we fund it. Vytalix is a group. Advisory and consulting — fixed-price sprints for trusts, ICBs, NGOs and health-tech founders — are the cash layer, invoicing from month two; the model puts them at roughly 60% of three-year revenue. E-learning follows. MaternaLink is the equity story, and it is priced on milestones, not multiples.

Where we are. Honestly: pre-formation. A concept, a prototype we're auditing, a government conversation whose outcome we don't know, and an IP settlement we're documenting. We have written down every claim we can't make yet.

The ask. £400k pre-seed, SEIS then EIS, to make seven things true in twelve months: IP owned, safety case, DPIA, DTAC readiness, MVP built and tested, a UK design partner, a Nigerian implementing partner. Those are the seed gates. Then a £2m seed on evidence in 2028.

We'd like your view on two things: who you know in maternity digital leadership, and what would make you say yes."

2. Investor executive summary

Item	Position (labelled)
Company	Vytalix — trading name (FACTS_BASE A1); legal entity "Vytalix Group Ltd" PROPOSED, England & Wales, incorporation pending name clearance (TO VALIDATE)
Stage	Pre-seed, pre-formation; solo founder; no revenue (KNOWN)

Item	Position (labelled)
Structure	One company, four divisions: Advisory · Consulting · Health Solutions (MaternaLink, in development) · E-Learning (02_GROUP_STRUCTURE Option A)
Product	MaternaLink (in development): non-diagnostic maternal care-coordination, communication, education and monitoring-support platform; v3+ research track towards validated risk insight, gated by data partnerships, ethics, validation and an MHRA SaMD route
Markets	UK (home; credibility) → Nigeria (volume; partner-first) → Kenya (third) (05_MARKET_STRATEGY)
Model	Services cash layer (day rates £1,200–£1,800; retainers £3,500–£5,000/month; sprints £9.5k–£28k) + product equity layer (UK site licence £8–£40/woman/yr; Africa programme licence £3–£12/woman/yr; pilots £40k–£150k/yr — all ESTIMATE)
Scenario revenue (Base, 08)	FY2027 £166k → FY2028 £488k → FY2029 £820k; EBITDA –£56k → –£190k → –£787k as the seed is deployed; not a forecast
Raise	Pre-seed £400k (SEIS £250k + EIS £150k, limits TO VALIDATE) at a £2.0m–£3.0m pre-money placeholder (ESTIMATE); target close month 4 of trading
Use	Product lead, MVP + pen-test, Clinical Safety Officer and advisers, compliance artefacts, co-design research, grant match, buffer (09 §3.1)
Next round	Seed £2.0m (2028) on: registered UK service evaluation, one paid pilot, safety-artefact pack, team of three+, Nigeria partner in Vytalix's name
What is not true today	No entity, IP, customers, pilots, partners, approvals, grants or advisers — each with a dated plan to change it

3. Investment thesis (five claims, each with its proof condition)

#	Claim	Why it may be true	What must be shown (and when)
1	The coordination-and-handover layer beside the maternity record is unowned	EPR vendors are record-centric (System C BadgerNet, K2 Athena, Magentus Euroking); Baby Buddy is information-only; African incumbents (PROMPTS, MomConnect, mDoc) are messaging- or coaching-led (11_COMPETITIVE_INTELLIGENCE §2)	Discovery interviews (15 UK, 10 Nigeria) confirming the gap in buyers' words — month 3; a design partner choosing to evaluate it — month 9–12
2	Policy pull is real and durable in the UK	Inquiry findings, the national maternity investigation, MBRACE-UK disparities, trust-level inequality obligations, the digital maternity record standard	Consulting engagements (C3, C8) sold to trusts/LMNS on exactly these obligations — first by month 3–6
3	One codebase can serve both an NHS trust and a low-connectivity programme	Offline-first, SMS/USSD/voice channels, DHIS2 export, NHS assurance artefacts — a combination no incumbent offers (11 §6.1 #3)	Prototype audit passed; MVP on the 07_TECHNOLOGY_ARCHITECTURE design; one low-connectivity field test — month 12–18
4	Services de-risk the venture	Founder-led sprints fund overhead from month 2; each engagement yields customer insight, artefacts and e-learning content; retainers are the highest-margin line in the model (08 §7)	≥ £15k/month services run-rate and two retainers before the seed

#	Claim	Why it may be true	What must be shown (and when)
5	Honesty is a competitive advantage in a category burnt by over-claiming	Trusts and specialist investors discount unvalidated apps; a published hazard log and service evaluation are rare (11 §6.1 #4)	Safety-artefact pack and a registered evaluation with a university-co-authored protocol — month 12–18

4. Problem

- **UK.** Public inquiries into maternity services (Ockenden 2022; Kirkup 2022; the 2025–26 national investigation naming 14 trusts — 05 §1.1) repeatedly identify failures to listen to women, to escalate concerns and to hand over between professionals and settings. MBRRACE-UK's 2025 report (deaths 2021–23) gives a maternal death rate of 12.82 per 100,000 maternities and finds Black women more than twice as likely to die as White women, with Asian women also at elevated risk (source and URL in 05 §1.1; cite the report itself in any external deck). England and Wales recorded 594,677 live births in 2024 (ONS, cited in 05 §1.1).
- **Nigeria.** The modelled maternal mortality ratio in 2023 was 993 per 100,000 live births, the highest in Africa (WHO/World Bank series via the compilation cited in 05 §2; TO VALIDATE against the primary report before external use). Nigeria is among the five countries with the largest absolute number of maternal deaths. The "three delays" — seeking, reaching and receiving care — are coordination failures, and connectivity gaps make offline-first design a requirement, not a feature.
- **The structural gap.** Care fails between people: between a woman and a changing set of professionals, between shifts, between community and hospital, and between a good idea and a safe, adopted product. Nobody owns that in-between space in either system.

5. Solution — MaternaLink (in development)

Intended use (PROPOSED v1): a care-coordination and communication platform for maternity services, designed to help women stay connected to their care team and help care teams hand over safely. It does not diagnose, predict, triage or score; it is designed to support, not replace, clinical judgement.

v1 scope: enrolment and consent · personalised contact schedules and reminders · clinically signed-off education in the woman's language · secure two-way messaging with a named human owner and an acknowledgement time · a self-recorded diary that is displayed, not interpreted · a care-team console · a structured handover summary · an escalation log · missed-contact lists · a programme dashboard. Channels: web app, SMS, WhatsApp, USSD and voice (roadmap v1.1–v1.2).

Four gates before anything is sold: IP documented · prototype audited · Clinical Safety Officer appointed and hazard log opened · DPIA completed (04_MATERNALINK_PRODUCT_STRATEGY).

The research track (v3+): the 2025 multimodal "Maternal Instability Score" concept is retained as R&D that advances only with data partnerships, ethics approval, clinical validation and a UK MDR / MHRA software-as-a-medical-device pathway. No clinical claim is made or implied before then.

6. Market (all ESTIMATE unless a source is named; sizes are for a coordination layer, not for maternity care)

Segment	Size logic	Value (ESTIMATE)	Vytalix's realistic reach by 2030
NHS trusts providing maternity (England)	~120 providers (ESTIMATE; TO VALIDATE) × £30k–£120k per site per year for a coordination layer	£3.6m–£14m per year ceiling (05 \$1.1)	5–10 trusts
UK private maternity, independent midwives, employers/benefits platforms	Small unit count; per-woman-per-episode £30–£80; £150–£300 per pregnancy via benefits platforms	Low single-digit £m per year	Credibility pilots; 1–3 contracts
Nigeria state/NGO/donor maternal programmes	~7m+ births per year nationally (ESTIMATE; TO VALIDATE); programmes typically donor-funded; comparable messaging programmes at under \$1 per mother (Jacaranda PROMPTS, Kenya — cited in 05 , 11)	At £3–£12 per woman per year, a 100,000-woman programme is £0.3m–£1.2m per year	1–2 state-scale or NGO programmes (10,000–100,000 women)
Kenya counties/NGOs (third market)	47 county buyers; strongest mobile rails; PROMPTS sets the price expectation	Similar per-woman logic	1 county/NGO programme
Services (Advisory, Consulting, E-Learning)	Trust/ICB improvement budgets, health-tech founders entering the NHS or Africa, NGOs, universities	Not sized top-down; bottom-up Base £867k Advisory + Consulting over three years (08)	The cash layer

What we do not claim: a TAM in the billions. The global digital-health and femtech market figures commonly quoted are not used here because they do not describe a buyer for this product. The honest framing is a bottom-up ceiling of a few £m per year in the UK and a per-woman programme market in Africa whose price point is the single largest unknown (08 \$7 #4).

7. Business model

Layer	Streams	Pricing (ESTIMATE; 01 BUSINESS MODEL)	Margin / timing
Cash	Founder-led advisory days; monthly retainers; ten fixed-price consulting sprints (discovery, business case, digital maternity readiness, DTAC readiness, UK/Africa market entry, health-inequalities review, evaluation); e-learning courses and licences from month 9	£1,200–£1,800/day; £3,500–£5,000/month; £9.5k–£28k per sprint; courses £49–£299, corporate licences £2.5k–£9k	75–87% gross margin; invoicing from month 2; 59% of Base three-year revenue
Equity	MaternaLink paid pilots; UK site licences per woman; Africa programme licences per woman; later EPR-integration and partner-access modules	Pilots £40k–£150k/yr; UK £8–£40/woman/yr (≈ £40k–£120k per trust at 3,000–5,000 births); Africa £3–£12/woman/yr	First pilot month 13 (Base); 26% of Base three-year revenue; 100% of the equity valuation
Bridge	Grants for the evidence programme	£75k (Base) to £400k (High-growth) over three years	Upside only; never load-bearing

8. Buildable competitive advantage (what can be true in 6–24 months; none is claimed today — 11 \$6.1)

- Vendor-neutral coordination layer with a full UK safety-artefact pack** (intended-use, DCB0129 hazard log, DPIA, DTAC self-assessment, Cyber Essentials Plus) — proof: artefacts reviewed by a contracted CSO and

DCB0160 run by one trust; month 9–12.

2. **Community-co-designed, multilingual content** (paid partnerships with Black maternal-health and community organisations; Yoruba, Hausa, Igbo, Pidgin) — proof: signed co-design agreements, clinical sign-off, testing with ≥ 20 women per market; month 6–12.
3. **Offline-first design that also meets NHS assurance** — proof: audited prototype, MVP on the reference architecture, one low-connectivity field test; month 12–18.
4. **Evidence-first positioning** — proof: registered service evaluation with a university-co-authored protocol; month 12–18.
5. **Partner-first Africa model** (implementing NGO, DHIS2 export, data owned by the state/NGO, local hosting option) — proof: an NGO cohort agreement and a documented complementarity conversation with mDoc; month 9–18.
6. **A services practice that feeds the product** — proof: two delivered engagements with consented case studies and one derived e-learning course; month 6–12.

What cannot be claimed now (and must never be): clinical outcomes, validated AI, NHS or government relationships, price advantage over donor-funded incumbents, any regulatory status, ownership of MaternaLink (until the deed is executed).

9. Technology (PROPOSED; 07_TECHNOLOGY_ARCHITECTURE)

- Web application plus SMS/WhatsApp/USSD/voice gateways; offline-first client with sync; role-based care-team console.
- Data: UK hosting in a UK region; Nigeria hosting option to meet NDPA expectations; DHIS2 export; FHIR read facade (v2); export interfaces to the three UK maternity EPRs (v2.2).
- Security and assurance: Cyber Essentials → Plus; DSPT; penetration test before any live use; DPIA; clinical-safety process (DCB0129) from the first line of production code.
- The existing prototype (Vercel, "operator access" registration) is of unknown maturity, hosting, security and provenance (VERIFIED existence only) and is being audited; it is a starting point or a reference, not the product.
- The AI concept stays in research until data, ethics and a regulatory route exist.

10. Traction — stated honestly

10.1 What we have (KNOWN / VERIFIED)

- A complete founder brief and a 20-document build (strategy, model, product, legal, sales, marketing, investor CRM) produced in September 2026.
- A 2025 concept deck for MaternaLink; a prototype dashboard (unaudited); an unsolicited proposal to Ekiti State Ministry of Health (March 2026; outcome unknown; pricing withdrawn from all future use).
- A researched CRM of ~300 publicly identifiable investors with fit and contactability scores, and a Top 25.
- A founder with demonstrated investor-CRM and outreach discipline in a separate venture (Kemek Enterprise; declared conflict).

10.2 What we are building (in progress from 14 September 2026)

- Legal entity, bank, accountant, insurance, ICO, trademark; SEIS/EIS advance assurance.

- MaternaLink IP settlement with David Agunede (assignment for vested equity preferred; hard stop day 60).
- v1 intended-use statement adopted; 2025 claims retired; prototype audit.
- Services engine: 40 contacts / 20 conversations / 5 discovery calls / 2 proposals per week from day 8.
- Advisory board recruitment (obstetrics, midwifery, NHS digital, Nigerian health-system, regulatory, health-tech scaling).

10.3 What's next — proof points and how each will be evidenced

Horizon	Proof point	Evidence produced (in the data room)
90 days (by 12 Dec 2026)	Incorporated; IP deed executed or clean-room decision minuted	Certificate; executed deed or board minute
90 days	First three signed services engagements (£15k–£30k contracted, ESTIMATE range)	SOWs; first paid invoices
90 days	15 UK and 10 Nigeria discovery interviews completed	Interview summaries with consent; synthesis memo
90 days	SEIS/EIS advance assurance submitted; data room v1; two named advisers	HMRC submission copy; data-room index; adviser letters
90 days	Prototype audit; Cyber Essentials; DPIA v0.1 and safety-case skeleton	Audit report; certificate; documents
90 days	Ekiti go/no-go decided; mDoc complementarity conversation held	Board minute; call note
90 days	One grant bid submitted; a sourced report published	Submission receipt; publication
180 days (by ~March 2027)	Pre-seed £400k closed (Base)	Share issue; SEIS/EIS compliance statements
180 days	MVP build under way with a product lead identified/hired (hire only after money)	Contract; repository; sprint reports
180 days	One UK maternity service in a signed unpaid design-partner conversation; one Nigeria NGO/implementing partner in discussion	Letters of intent (if any) — described as "in discussion" until signed
180 days	Two retainers active; ≥ £10k/month services run-rate	Invoices; KPI dashboard
180 days	CSO contracted; hazard log open; DTAC self-assessment started	Contract; hazard log

Numbers received in the base case for the first 90 days: **£0 of external funding** (18_90_DAY_EXECUTION_PLAN).

11. Roadmap (PROPOSED; product/MATERNALINK_ROADMAP.md)

Period	Product	Evidence	Company
Q4 2026	Discovery; build weeks 1–6 (after IP and funding decisions)	Discovery interviews	Entity, IP, claims, sales engine, pre-seed preparation
Q1–Q2 2027	MVP complete; alpha; v1.0 live in UK service evaluation; v1.1 WhatsApp/Yoruba/voice	UK service-evaluation protocol and start (n 50–150)	Pre-seed closed; product lead hired; CSO; two e-learning courses (month 9)
Q3–Q4 2027	v1.2 USSD, DHIS2 export; v2.0 FHIR read facade; device-qualification decision	Nigeria pilot design, ethics, start (1–2 LGAs, 500–2,000 women); UK feasibility study prep	First paid pilot (Base month 13 = Jan 2028); grant decision

Period	Product	Evidence	Company
2028	v2.1–v2.3 partner access, further languages, EPR export interfaces, hardening; v3 research design	UK feasibility (2 sites); multi-site formal evaluation under way; Nigeria pilot reporting	Seed £2.0m (Base month 20 = Aug 2028); team of 7–8
2029	Scale features; integrations; v3 retrospective modelling under approvals	Evaluation results; process-outcome language earned	2–3 UK sites; one state-scale agreement; Series A decision (month 30–42)

12. Funding requirement and use of funds

Round	Amount	When (Base)	Gates (must be TRUE)	Use of funds
Pre-seed	£400k (SEIS £250k + EIS £150k; limits TO VALIDATE); range £250k–£500k	Month 4 (April 2027)	Entity; IP deed; claims retired; advance assurance applied for; one signed engagement; data room; two advisers	Product lead (~£55k in-year); MVP + pen-test (~£90k); CSO and advisers (~£25k); compliance (~£15k); co-design research (~£30k); legal/IP (~£10k); grant match (~£30k); founder top-up (~£25k); buffer (~£120k)
Seed	£2.0m; range £1.5m–£3m	Month 20 (Aug 2028)	Registered UK evaluation; paid pilot; artefact pack; team ≥ 3; services ≥ £15k/month; Nigeria partner in Vytalix's name; clean cap table	Team (~£560k); implementation/tech (~£500k); v1.1–v2.2 build (~£250k); evidence programme (~£300k); compliance (~£90k); marketing/e-learning (~£150k); buffer (~£150k)
Series A	£6m–£12m	Month 30–42 decision	ARR £0.4m–£0.9m or ≥ 3 UK sites + one state programme; formal evaluation results; conversions; team ~10; Africa WTP validated	Scale to 5–10 trusts and 2–3 programmes; in-country teams; EPR integrations; regulatory/QA; v3 research (ring-fenced)

Cash logic: Base peak cash need £1.06m to month 36 (08 \$4); rounds sized ~1.5–2× need to provide 18–24 months' post-round runway; grants are upside only.

13. Exit scenarios (types only; no timing, valuation or probability is promised)

Acquirer type	Why they might buy	What would have to be true
UK maternity-record / hospital-software vendors (the three maternity EPR vendors, or larger clinical-software groups)	Patient-facing coordination layer; integration footprint; inequality and safety narrative	≥ 5 trust deployments; DTAC; integration with two of three EPRs
Women's-health and virtual-maternity platforms (US/European) seeking NHS and Africa footprints	Regulatory-ready UK asset; low-connectivity capability; multilingual content	Evidence and contracts in both systems
Health-system consultancies and advisory groups	The services practice plus a product; UK–Africa corridor expertise	A profitable services business with references and a team
Impact-oriented strategics: telecoms, insurers/HMOs, pharma women's-health units	Distribution of a coordination layer to their maternal populations	State-scale programme; per-woman economics validated
Continued independence / secondary	The High-growth scenario is cash-generative from month 31	Sustained EBITDA and licence growth

Founder position: build to be worth owning; keep the cap table and IP clean so every option remains open.

14. Fifteen hard investor questions — prepared answers

1. **"Do you own MaternaLink?"** — Not yet, and we say so. The IP relationship with David Agunede is undocumented. We are executing an assignment-for-vested-equity deed (8–12% PROPOSED, four-year reverse vesting, warranties, IP audit first), with an exclusive licence as fallback and a clean-room rebuild if there is no agreement by day 60. No investor money is taken before this is resolved.
2. **"Your 2025 deck says the AI predicts pre-eclampsia and sepsis. Where's the data?"** — There is none, and that deck is retired. v1 is non-diagnostic care coordination with no predictive features. The AI concept is a v3 research track that advances only with data partnerships, ethics approval, prospective validation and an MHRA SaMD route. We will not make a clinical claim before that.
3. **"Why should I fund a consultancy?"** — You are not. You are funding milestones that convert MaternaLink into a licensable asset; the services business pays the overhead so your money goes to product, safety artefacts and evidence. The model prices the equity on milestones, not on services multiples. If you want a pure product company, we are the wrong pre-seed and the right seed.
4. **"What about the Ekiti proposal — £300 per woman and £36m?"** — It was sent under "Maternal Link" before Vytalix's involvement was structured; the outcome is unknown; the pricing is withdrawn. Comparable programmes run at under \$1 per mother; our model uses £3–£12 per woman per year for Africa and treats willingness to pay as the largest unknown. Ekiti has a funded incumbent (mDoc / MSD for Mothers); our options are partner, move state or stop — decided by day 60.
5. **"Who is the customer and who pays?"** — UK: the trust (Head/Director of Midwifery, digital lead), via a design partnership then a paid pilot then a site licence; Africa: the implementing NGO, state programme or donor, per woman per year. Services: trusts/ICBs, NGOs and health-tech founders, from month two.
6. **"Why won't the EPR vendors just add this?"** — They may add features; they do not build community-co-designed, multilingual, offline-first coordination with independent evidence, and trusts value vendor neutrality. Our answer is integration-ready and evidence-generating, not "better app". If a vendor prefers to buy rather than build, that is an exit type.
7. **"Solo founder — what happens if you are ill or distracted?"** — This is the binding constraint and the model says so. Mitigations: a product lead from pre-seed funds, a contracted CSO, a named advisory board, a written $\geq 70\%$ time-allocation rule with the other venture declared, and a hard stop on billable days.
8. **"What's the regulatory position?"** — v1 is designed to fall outside medical-device classification (intended use: coordination and communication; no diagnosis, triage or scoring) — TO VALIDATE with regulatory counsel and documented in a qualification memo. UK: DCB0129/0160, DPIA, DTAC, DSPT, Cyber Essentials. Nigeria: NDPA registration where required, NHREC for any evaluation. Any v2 monitoring feature goes through a documented device decision (gate G8).
9. **"How will you get an NHS trust to evaluate it?"** — An unpaid design partnership via Health Innovation Networks, LMNS leads and clinical advisers; ten approaches; we arrive with the artefacts trusts usually have to ask for. If nobody engages by month 9, we go private maternity or Africa-first.
10. **"What's the SEIS/EIS status?"** — Applied for [or: to be applied for in month 1–2], not approved. We expect to structure £250k SEIS then £150k EIS (limits TO VALIDATE). The risk is HMRC scrutiny of a services-heavy trade; the application is framed around the R&D programme.
11. **"What does the £400k actually buy me?"** — Seven dated gates: IP owned; safety case; DPIA; DTAC readiness; MVP built and pen-tested; UK design partner; Nigeria implementing partner. Hit them and the seed

is priced on evidence; miss them and you will know why, by month.

12. **"What are the unit economics?"** — UK: £8–£40 per woman per year against ~£4 hosting/support (ESTIMATE); Africa: £3–£12 against £2–£3 hosting/SMS/support; pilots £40k–£150k at 40–50% delivery cost initially; services 75–87% gross margin. All scenario values; none validated by a customer.
13. **"When do you break even?"** — Services break even by month 3–4. The company does not break even inside 36 months in Conservative or Base because it chooses to invest in the product; High-growth reaches sustained EBITDA in month 31. Break-even is a choice about scaling speed, and we will not scale the MaternaLink cost base until go-live is contractually dated.
14. **"What kills this?"** — No IP agreement; no design partner; Africa willingness to pay below £3; the pre-seed slipping past month 6; a predictive feature shipping without a device pathway. Each has a kill signal and a fallback written down (08 \$8; 09 \$6).
15. **"Why you?"** — The honest answer is that the founder's credentials are being documented and will be evidenced by delivered engagements, not asserted. What is already visible: a rigorous, stage-honest build, an investor-outreach discipline proven elsewhere, and a willingness to write down every claim we cannot make. The clinical, technical and regulatory leadership comes from the advisory board and the first hires, and we name them only when they have consented.

Priorities / Risks / Next actions

Priorities

1. Use the pitches only after the pre-conditions are met; until then, conversations are "advice and introductions", never "a raise".
2. Keep \$10 the single source of traction language; update it every Friday from the KPI dashboard and never let a deck run ahead of it.
3. Practise the 15 answers until the IP, claims, Ekiti and solo-founder answers are reflexive — those four decide whether a second meeting happens.
4. Replace every ESTIMATE in \$6 with a cited primary source before external use; cite MBRRACE-UK 2025 and ONS directly on any slide that uses them.
5. Convert the 90-day proof points into data-room artefacts as they land, so the seed narrative writes itself from evidence.

Risks

- A listener remembers the 2025 deck or the Ekiti figures — pre-empt with the retirement statement in the first minute.
- Market sizing invites a "too small" objection — hold the bottom-up framing and the per-woman programme logic; do not reach for a global TAM.
- Traction language drifts to "partner", "pilot", "approved" — the FACTS_BASE \$4 vocabulary is mandatory in every asset.
- Valuation placeholder is challenged — treat it as a range to be set by the lead, not a number to defend.
- Founder credentials cannot yet be evidenced — do not over-claim the "UK–Africa corridor" positioning until engagements support it.

Next actions (owner: Fundraising Director)

- Week 1: record the 2-minute script; test on two friendly listeners; tighten to under two minutes.
- Week 2: build the one-pager and deck from this document ([/assets/](#)); assemble the 15-answer sheet as a printed page for meetings.
- Week 4: first valuation sounding with three investors after the IP deed and advance-assurance submission.
- Week 8: refresh §10 with the first 60 days of actuals; re-issue the deck as v1.1.
- Month 4: post-close review — what worked in the pitch, which questions were asked that are not in §14, add them.